

Paediatric Emergency Intubation Checklist

Step 1: PLAN AHEAD

Brief child / parents

Difficulty expected?

History of difficult intubation/ventilation

Anatomical

Mouth / Jaw / Neck

Obstructions:

Mass, FB, oedema

Syndromes

Physiological

↓O₂ / ↓BP / ↓pH / CCF

☐ YES / NO

Right team?

ED Consultant 8021

Anaesthesia 6111

Paeds Anaesthesia Cons 8448 (Bleep

8am - 5pm) or switch out of hours

PICU 6890/ ENT 6075

☐ YES / NO

Right location?

Is this the best location?

Transfer to PICU / Theatre?

☐ YES / NO

Step 2: EQUIPMENT CHECK

Monitoring

- ☐ ETCO₂
- ☐ SpO₂ + tone, not on NIBP limb
- ☐ NIBP every 1-2min
- ☐ ECG

Airway

- ☐ Mask + OPA / NPA
- ☐ T piece and oxygen
- ☐ Suction- yankauer & catheters
- ☐ Laryngoscopes x2 working
+/- VideoLaryngoscope
- ☐ ETT +/- sizes + syringe
- ☐ Bougie / Stylet / Magill
- ☐ LMA
- ☐ Tapes

Drugs (see overleaf)

- ☐ Allergies?
- ☐ Induction / analgesia
- ☐ Muscle relaxant
- ☐ Rescue meds
(Atropine, adrenaline, inotropes)
- ☐ Fluid boluses
- ☐ Maintenance sedation

Step 3: PATIENT CHECK

☐ WETFLAG

☐ IV access x 2

☐ Optimal physiology

- ☐ Pre-oxygenation - aim 3 mins
- ☐ Haemodynamic status - fluids /
inotropes required?

☐ Optimal Positioning

- ☐ Shoulder roll/pillow?
- ☐ Appropriate bed height?

☐ NGT inserted / aspirated

- repeat throughout mask ventilation

Step 4: TEAM CHECK

Roles

- ☐ Confirm names
- ☐ Team Leader
- ☐ Intubator 1 + 2
- ☐ Airway assistant
- ☐ Drugs
- ☐ NGT aspiration
- ☐ Cricoid? MILS?
- ☐ Runner

Plan for failure (see overleaf)

- ☐ Plans A to D inc FONA
- ☐ Plan for physiological collapse
- ☐ Plan for Help

Any questions / comments?

CONFIRM SUCCESS

ETCO₂ x 6 waves (if in doubt take out)

Check & Secure ETT / Start ventilator /
Start sedation / CXR - ETT tip at T2/3

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EQUIPMENT

Cuffed ETT

Term neonate 3.0

> 5 months 3.5

> 1 year (age/4 +3.5)

Oral Length(cm) = ETT ID x3 or age/2
+12

Nasal (cm) = age/2 + 15

Uncuffed ETT if <3kg

Bougie (Ch)

5 - ETT <3.5

10 - ETT 4-5

15 - ETT >6.0

LMA

1 - <5kg

1.5 - 5-10kg

2 - 10-20kg

2.5 - 20-30kg

3 - 30-50kg

4 - >50kg

NGT

Size in Ch = ETT ID x2

Length = NEX measurement

Suction catheter

Size in Ch = ETT IDx2

DRUGS

Suggested regimen

Fentanyl 1-2mcg/kg

Ketamine 1-2mg/kg

Rocuronium 1mg/kg

If haemodynamically unstable

Use reduced dose induction + titrate

Consider omitting fentanyl

Ketamine 0.2-0.5mg/kg

Rocuronium 1mg/kg

Have Resuscitation drugs available:

Atropine 20mcg/kg

ALS adrenaline 10mcg/kg

Bolus adrenaline 1mcg/kg

Bolus phenylephrine 2mcg/kg

Bolus metaraminol 5mcg/kg

Consider peripheral inotropes

Adrenaline 0.02 - 0.5mcg/kg/min

Noradrenaline 0.02 - 0.5mcg/kg/min

Dobutamine 5-20mcg/kg/min

Prostin 5 - 100 ng/kg/min

Fluids

10ml/kg Crystalloid boluses

DAS GUIDELINES

Difficult Mask Ventilation



Difficult Tracheal intubation



Can't Intubate Can't Ventilate



VENTILATION

Suggested settings:

Tidal volume 6-8ml/kg

Limit Ppeak <30cmH2O

PEEP 4-8cmH2O

iTime 0.6-0.9s

Rate according to age

Minute Ventilation 100-300ml/kg

FiO2 for SpO2 94-98%

Troubleshooting DOPES

Displacement

Obstruction

Pneumothorax

Equipment

Stomach / Suction / Stacking

Unusual Dx - e.g tracheal stenosis